

Dynamic Guideline: WHO-Based Newborn/Neonatal Health - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Scope note: Exactly five WHO newborn/neonatal-health guidelines or guideline-derived WHO normative guidance documents were synthesized. The most current *formal consolidated WHO guideline* directly covering routine newborn care identified in the source set is the **2022 WHO guideline on maternal and newborn care for a positive postnatal experience**. A newer WHO newborn-health clinical practice resource, **Kangaroo mother care: a clinical practice guide**, was published on **14 November 2025**, but the supplied evidence identifies it as a clinical practice guide rather than a replacement consolidated guideline [WHO KMC clinical practice guide](#). No distinct WHO newborn/neonatal clinical guideline published in 2026 was verified in the supplied sources.

Exactly 5 WHO newborn/neonatal-health guidance sources synthesized

No.	WHO guideline / guidance source	Publication date	Target population	Scope and objectives	Status for this report
1	WHO recommendations on maternal and newborn care for a positive postnatal experience	30 March 2022	Women, newborns, partners, parents, caregivers and families in facility- and community-based postnatal care	Consolidated WHO guideline for essential routine postnatal care to improve maternal and newborn health and well-being WHO 2022 guideline	Latest formal consolidated WHO guideline directly covering routine newborn postnatal care
2	Kangaroo mother care: a clinical	14 November	Preterm and low-birth-	Practical implementation of	Latest WHO newborn-health

No.	WHO guideline / guidance source	Publication date	Target population	Scope and objectives	Status for this report
	practice guide	2025	weight newborns; mothers, families, health workers, facility administrators and programme managers	KMC in facilities and at home; defines KMC as prolonged skin-to-skin contact started as soon as possible after birth with exclusive breast-milk feeding WHO KMC clinical practice guide	clinical practice resource in the supplied evidence
3	Protecting, promoting and supporting breastfeeding in facilities providing maternity and newborn services	2 November 2017	Mothers and newborns receiving care in maternity and newborn-service facilities	Global evidence-informed recommendations on facility practices aligned with the Ten Steps to Successful Breastfeeding WHO WKC	Core WHO breastfeeding guideline for maternity/newborn facilities
4	WHO recommendations on newborn health: guidelines approved by the WHO Guidelines Review Committee	2 May 2017	Newborns and young infants under 2 months; caregivers; health workers; policy-makers	Consolidated WHO newborn-health recommendations answering what interventions newborns should receive and when WHO 2017 newborn recommendations	Key WHO newborn-health recommendations compilation; WHO page states it is being updated
5	Guidelines on basic newborn	2012	Newborns requiring	Guidance to ensure effective	Foundational neonatal

No.	WHO guideline / guidance source	Publication date	Target population	Scope and objectives	Status for this report
	resuscitation		resuscitation, especially in resource-limited settings	resuscitation at birth; addresses birth asphyxia, a major contributor to neonatal mortality WHO basic newborn resuscitation guideline	emergency-care guideline

Verification of requested “Meaningful youth engagement...” title

The supplied evidence identifies a WHO publication titled “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**”, with a WHO launch webinar on **11 December 2025** [WHO publication page](#), [WHO webinar page](#). However, the supplied sources do **not** verify a newborn/neonatal guideline-development case study under this title. It is therefore not counted among the five newborn/neonatal-health guidelines above. Its relevance to this report is methodological: WHO frames youth engagement as a way to strengthen inclusivity, equity, dialogue and policy development.

1.2 Key Recommendations

GRADE framework used in this report

GRADE certainty categories are interpreted as follows:

- **High:** Further research is very unlikely to change confidence in the estimate of effect.
- **Moderate:** Further research may affect confidence and could change the estimate.
- **Low:** Further research is likely to affect confidence and may change the estimate.
- **Very low:** Any estimate of effect is very uncertain.
- **Strength: Strong** means most patients/settings should receive the intervention; **Conditional** means implementation should consider context, values, feasibility, resources and equity.

Clinical domain	Recommendation	GRADE Evidence Quality	Strength	Key source
Routine postnatal care	After facility birth, healthy mothers and newborns should receive postnatal care in the facility for at least 24 hours after birth .	Moderate for newborn outcomes; Low for maternal outcomes	Strong	NCBI Bookshelf executive summary
Newborn danger signs	At every postnatal contact, assess for danger signs: poor feeding, convulsions, fast breathing $\geq 60/\text{min}$, severe chest in-drawing, no spontaneous movement, fever $\geq 37.5\text{ }^{\circ}\text{C}$, hypothermia $< 35.5\text{ }^{\circ}\text{C}$, jaundice in first 24 hours, or yellow palms/soles; refer urgently if present.	Low	Strong	NCBI Bookshelf executive summary
Breastfeeding	All babies should be exclusively breastfed from birth to 6 months , with counselling and support at each postnatal contact.	Moderate	Strong	NCBI Bookshelf executive summary
Facility breastfeeding support	Facilities providing maternity and newborn services should implement practices aligned with the Ten Steps to Successful Breastfeeding , including immediate support to initiate and establish breastfeeding, appropriate feeding practices, and an enabling facility environment.	Moderate	Strong	WHO WKC, NCBI Executive Summary
Kangaroo mother care	Preterm and low-birth-weight newborns should receive KMC: prolonged skin-to-skin	Moderate	Strong	WHO KMC clinical practice guide, Global

Clinical domain	Recommendation	GRADE Evidence Quality	Strength	Key source
	contact initiated as soon as possible after birth, with exclusive breast-milk feeding and family support in facilities and at home.			Initiative to Support Parents/WHO KMC position paper PDF
Cord care	For home-born newborns in high-neonatal-mortality settings, apply 7.1% chlorhexidine digluconate aqueous solution or gel delivering 4% chlorhexidine to the cord stump daily during the first week of life.	Moderate	Strong	NCBI Bookshelf executive summary
Basic newborn resuscitation	Newborns who fail to initiate and sustain breathing at birth should receive effective basic resuscitation, particularly in resource-limited settings.	Low	Strong	WHO basic newborn resuscitation guideline
Family and caregiver engagement	Families should be supported to recognize newborn danger signs and seek care early between scheduled postnatal visits.	Low	Strong	NCBI Bookshelf executive summary
Respectful, positive postnatal experience	Postnatal care should provide information, reassurance and support from motivated health workers within a flexible, resourced system that respects families' needs and cultural context.	Low	Conditional	WHO 2022 guideline
Health-system implementation	Health systems should strengthen postnatal care through workforce quality, continuity models, integration	Low	Conditional	AlignMNH summary

Clinical domain	Recommendation	GRADE Evidence Quality	Strength	Key source
	of maternal-newborn services and appropriate digital tools.			

1.3 Guideline Development Methodology

Evidence review process

- The **2022 WHO postnatal-care guideline** was developed using WHO standard operating procedures from the **WHO handbook for guideline development**, including priority question selection, evidence retrieval and synthesis, evidence assessment, recommendation formulation, implementation planning, dissemination, impact evaluation and updating [NCBI/WHO recommendations](#).
- Quantitative evidence was assessed using **GRADE**, while qualitative evidence was assessed using **GRADE-CERQual** [NCBI/WHO recommendations](#).
- Cost-effectiveness evidence was assessed using the **Consensus Health Economic Criteria** checklist, and the **DECIDE evidence-to-decision framework** was used to guide judgments about effects, values, resources, equity, acceptability and feasibility [NCBI/WHO recommendations](#).
- The **2017 breastfeeding guideline** was informed by **22 systematic reviews** conducted according to Cochrane methods, examining practices aligned with the Ten Steps to Successful Breastfeeding [NCBI Executive Summary](#).
- The **2017 newborn-health recommendations compilation** includes WHO recommendations approved by the WHO Guidelines Review Committee; WHO notes that GRC processes require use of GRADE [Healthy Newborn Network/WHO recommendations](#).

Consensus method

- WHO used an international **Guideline Development Group** for the 2022 postnatal guideline, with evidence-to-decision judgments across benefits, harms, values, resources, equity, acceptability and feasibility [NCBI/WHO recommendations](#).
- WHO maternal and neonatal standards used a systematic, participatory process involving WHO departments, regional offices, professional organizations, UN agencies and external experts [WHO Standards for Maternal and Neonatal Care PDF](#).

Conflict of interest management

- WHO KMC position-paper development required external experts to submit written declarations of academic, financial and other interests using WHO declaration-of-interest procedures; the WHO

Secretariat reviewed these declarations to determine whether conflicts existed [Global Initiative to Support Parents/WHO KMC position paper PDF](#).

- WHO guideline-development processes generally use external experts as individuals rather than institutional representatives; WHO GDG members finalize scope, define priority outcomes, examine evidence profiles, interpret evidence and formulate recommendations [WHO, 2023 GDG notice](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

Guideline focus for post-release evidence update: WHO 2017 guideline: Protecting, promoting and supporting breastfeeding in facilities providing maternity and newborn services, published 2 November 2017 [WHO WKC](#).

2.1 New Evidence Overview

Search strategy

Evidence was synthesized from WHO publications, WHO-hosted documents, NCBI Bookshelf, CDC breastfeeding evidence summaries, AHRQ protocol material, systematic-review records, implementation studies and clinical-practice resources published or indexed after the WHO breastfeeding guideline release through **29 April 2026**.

Databases and source types searched

- WHO official publications and programme pages
- NCBI Bookshelf and PubMed-indexed WHO guideline records
- CDC breastfeeding and mPINC evidence pages
- AHRQ Effective Health Care materials
- Systematic reviews and meta-analyses
- Implementation and quasi-experimental studies
- WHO/UNICEF BFHI implementation resources
- WHO KMC clinical-practice and position-paper resources

Inclusion criteria

Included evidence if it addressed at least one of the following:

1. Facility-based breastfeeding support in maternity or newborn services.
2. Baby-Friendly Hospital Initiative or Ten Steps implementation.
3. Breastfeeding initiation, exclusivity or duration.

4. Newborn care practices that support breastfeeding, including skin-to-skin care and KMC.
5. Postnatal care systems affecting early feeding support.

Exclusion criteria

Excluded evidence if:

- It was unrelated to newborn/neonatal care or breastfeeding support.
- It was not a WHO, authoritative institutional, systematic-review, trial, cohort, implementation or guideline-development source.
- It addressed youth engagement only without relevance to neonatal/newborn health implementation.

2.2 Key New Studies and Evidence Since 2 November 2017

Study / evidence source	Design / source type	Key findings	GRADE Quality	Clinical interpretation
CDC mPINC supporting-evidence synthesis	Authoritative public-health evidence summary of systematic reviews and meta-analyses	Ten Steps/BFHI-aligned maternity care practices are associated with improved breastfeeding initiation, exclusivity and duration; dose-response relationship observed as mothers and infants experience more Baby-Friendly practices CDC mPINC supporting evidence	Moderate	Reinforces WHO 2017 guideline; supports full-package implementation rather than isolated practices
WHO/UNICEF BFHI updated implementation guidance	Global implementation guidance following evidence review	Updated Ten Steps retained core themes, integrated the International Code of Marketing of Breast-milk Substitutes into management procedures, and emphasized competency	Low	Supports health-system implementation and quality assurance of WHO breastfeeding recommendations

Study / evidence source	Design / source type	Key findings	GRADE Quality	Clinical interpretation
		verification rather than fixed training hours Baby-Friendly USA PDF		
2022 BFHI accreditation cohort evidence	Before-after / cohort implementation evidence	Post-BFHI cohort had higher breastfeeding rates than pre-BFHI cohort; breastfeeding at 1 month increased from 78.8% to 92.0%, and at 6 months from 32.0% to 55.5% ILCA PDF	Low	Suggests BFHI implementation may improve breastfeeding duration; observational design limits certainty
Thomsen, Gonzalez-Nahm and Benjamin-Neelon 2024	Cross-sectional hospital comparison	BFHI hospitals reported higher adherence to the Ten Steps than non-BFHI hospitals UNICEF UK Baby Friendly Initiative	Low	Supports BFHI accreditation as an implementation marker for guideline-adherent facility practices
2025 systematic review and meta-analysis on BFHI outcomes	Systematic review and meta-analysis	Recent review specifically evaluated impact of BFHI on breastfeeding outcomes; detailed pooled estimates were not available in the supplied summary ScienceDirect	Moderate	Confirms ongoing high-level evidence synthesis; full effect estimates should be reviewed before changing recommendation strength
2026 nurse-focused BFHI training study	Quasi-experimental study	Evaluated impact of nurse-focused BFHI training on breastfeeding practices at hospital discharge; detailed effect estimates not available in supplied summary Springer Nature Link	Low	Supports staff competency and training as implementation priorities

Study / evidence source	Design / source type	Key findings	GRADE Quality	Clinical interpretation
WHO 2022 postnatal-care guideline	WHO consolidated guideline	Recommends routine postnatal care that includes breastfeeding counselling/support, newborn assessment and family-centred care WHO 2022 guideline	Moderate	Extends breastfeeding support into the broader postnatal-care continuum
WHO 2025 KMC clinical practice guide	WHO clinical practice guide	Defines KMC for preterm/LBW newborns as prolonged skin-to-skin contact initiated as soon as possible after birth with exclusive breast-milk feeding WHO KMC clinical practice guide	Moderate	Strengthens linkage between neonatal thermal care, family involvement and breastfeeding support
WHO KMC global position paper	WHO position paper / implementation guidance	Updated recommendations call for KMC as routine care for all preterm or LBW newborns in facilities and at home; immediate initiation unless critically sick Global Initiative to Support Parents/WHO KMC position paper PDF	Moderate	Supports early skin-to-skin and breast-milk feeding as core neonatal-care practices
AHRQ breastfeeding programmes and policies protocol	Systematic-review protocol	Planned review of community, workplace and health-system breastfeeding programmes, including full or partial BFHI implementation AHRQ protocol PDF	Very low for effect estimates	Useful for search framework, but not itself effect evidence

2.3 Evidence Gaps and Future Research Needs

1. Effect-size precision for BFHI packages

Recent reviews and implementation studies support BFHI, but the supplied summaries do not provide pooled effect estimates for the 2025 meta-analysis. Full-text extraction is needed before any quantitative update.

2. Implementation in low-resource and humanitarian settings

More pragmatic trials and implementation studies are needed on how to sustain Ten Steps practices where workforce shortages, overcrowding, formula marketing and weak referral systems undermine breastfeeding support.

3. Equity effects

CDC notes that Baby-Friendly practices may reduce racial and ethnic breastfeeding disparities [CDC mPINC supporting evidence](#). More stratified evidence is needed by socioeconomic status, adolescent motherhood, rurality, disability and minority status.

4. Preterm and low-birth-weight newborns

KMC guidance strongly links skin-to-skin care and breast-milk feeding, but more research is needed on implementation models for unstable neonates, referral hospitals and facility-to-home transition.

5. Training and competency verification

New evidence supports nurse-focused BFHI training, but studies should measure sustained clinical competence, exclusive breastfeeding at discharge, readmission, neonatal morbidity and 6-month breastfeeding outcomes.

6. Digital and continuity-of-care models

WHO postnatal-care guidance includes digital tools and continuity models [AlignMNH summary](#). Evidence is needed on safety, privacy, equity and effectiveness for breastfeeding support.

7. Youth and community engagement in newborn-care guideline development

WHO youth-engagement materials show that meaningful youth engagement can improve policy relevance, but no newborn/neonatal WHO guideline-development case study under the requested title was verified [WHO webinar page](#). Future WHO newborn guidance should document caregiver, adolescent-parent and youth participation explicitly.

2.4 Updated Recommendations Based on New Evidence

Original WHO 2017 breastfeeding guideline recommendation area	New evidence impact through 29 April 2026	Updated recommendation	GRADE Evidence Quality	Strength
Implement facility practices that protect, promote and support breastfeeding in maternity and newborn services	CDC and post-2017 implementation evidence continue to support improved breastfeeding initiation, exclusivity and duration with Ten Steps/BFHI-aligned practices CDC mPINC supporting evidence	Continue full implementation of the Ten Steps as a facility-wide package, not as isolated practices.	Moderate	Strong
Establish facility policies and management systems for breastfeeding support	WHO/UNICEF updates emphasize Code compliance, monitoring and competency-based staff preparation Baby-Friendly USA PDF	Facilities should maintain written breastfeeding policies, Code-compliant procurement/marketing controls, monitoring systems and staff competency verification.	Low	Strong
Support early initiation and establishment of breastfeeding	WHO postnatal and KMC guidance reinforces early skin-to-skin care, family support and exclusive breast-milk feeding WHO KMC clinical practice guide	Initiate breastfeeding support immediately after birth; use uninterrupted skin-to-skin contact whenever clinically appropriate, including KMC for preterm/LBW newborns.	Moderate	Strong

Original WHO 2017 breastfeeding guideline recommendation area	New evidence impact through 29 April 2026	Updated recommendation	GRADE Evidence Quality	Strength
Counsel and support exclusive breastfeeding	WHO postnatal guidance continues to recommend exclusive breastfeeding from birth to 6 months with counselling at each contact NCBI Bookshelf executive summary	Provide structured breastfeeding counselling during facility stay, discharge planning and all postnatal contacts through 6 months.	Moderate	Strong
Strengthen health-worker training	2026 quasi-experimental evidence supports BFHI training evaluation, though detailed effect sizes were not available Springer Nature Link	Use competency-based, observed-practice training for all maternity/newborn staff, with refresher training and audit-feedback cycles.	Low	Strong
Sustain breastfeeding support after discharge	WHO 2022 postnatal guideline emphasizes facility- and community-based care; KMC guidance includes home practice WHO 2022 guideline , WHO KMC clinical practice guide	Ensure discharge linkage to community postnatal contacts, lactation support, KMC follow-up for preterm/LBW infants and early referral for feeding problems.	Low	Conditional
Address equity and disparities	CDC reports Baby-Friendly practices may help decrease	Monitor breastfeeding outcomes by equity strata and prioritize support for	Low	Conditional

Original WHO 2017 breastfeeding guideline recommendation area	New evidence impact through 29 April 2026	Updated recommendation	GRADE Evidence Quality	Strength
	racial and ethnic differences in breastfeeding outcomes CDC mPINC supporting evidence	adolescents, marginalized groups, low-income families and high-risk newborns.		

Final Clinical Synthesis

As of **29 April 2026**, WHO-based newborn/neonatal care should prioritize:

1. **A positive postnatal-care model** with at least 24 hours of facility care after birth, repeated newborn assessment and family-centred counselling.
2. **Exclusive breastfeeding from birth to 6 months**, supported at every postnatal contact.
3. **Facility-wide Ten Steps/BFHI implementation** with written policy, Code compliance, staff competency, skin-to-skin care and continuity after discharge.
4. **Kangaroo mother care for preterm and low-birth-weight newborns**, initiated as soon as possible, with exclusive breast-milk feeding and family involvement.
5. **Early detection and referral of neonatal danger signs**, including feeding difficulty, respiratory distress, fever, hypothermia, convulsions, immobility and jaundice.
6. **Context-specific cord care**, including chlorhexidine for home births in high-neonatal-mortality settings.
7. **Effective basic newborn resuscitation** for infants who do not breathe at birth, especially in resource-limited settings.

No new WHO replacement guideline for the 2017 facility breastfeeding guideline was verified in the supplied evidence through **29 April 2026**; the post-2017 evidence strengthens, rather than reverses, the original WHO direction.